NCCP 00794 · VERSION 3

Sacituzumab govitecan (Trodelvy)

Two treatment visits in each three-week cycle: Day 1, then Day 8. There is no planned infusion on Day 15. The next cycle starts after Day 21 if the team confirms it is appropriate.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Low neutrophils and serious infection, including febrile neutropenia

☐ Diarrhoea, dehydration and electrolyte loss; sometimes kidney injury

☐ Nausea, vomiting and reduced appetite

☐ Fatigue and anaemia

☐ Hair loss

☐ Infusion reactions, including rare life-threatening reactions

☐ Potential harm to an unborn baby and effects on fertility

Blood-count rules differ between Day 1 and Day 8. Known reduced UGT1A1 activity may increase side effects, but routine genotype testing is not required by the EU product information. Important differences between the NCCP and current EU dose-modification wording are flagged for Consultant and oncology-pharmacy review; patients should never change the dose themselves. Discuss fertility before treatment. Effective contraception is needed during treatment and for six months after the final dose for women who could become pregnant, and for three months for male patients with female partners who could become pregnant. Do not breastfeed during treatment; the EU product information also advises against breastfeeding for one month after the last dose.

Individual benefit, alternatives and concerns

Agreed checks and next review

Before the first treatment: full blood count, kidney and liver blood tests, glucose and electrolytes including magnesium, calcium and phosphate. Pregnancy testing is needed where relevant. ECG is added if clinically indicated. The team checks infection, medicines, previous low blood counts and whether preventive G-CSF injections are appropriate. Blood count, kidney and liver tests are checked before each cycle; the neutrophil count must also meet the separate Day 8 threshold before that infusion. Glucose and the listed electrolytes are checked every second cycle. Pregnancy testing is repeated before each cycle where relevant. Review diarrhoea, hydration, nausea, oral intake, fever and infection before each infusion. Scans follow the individual response-assessment plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.